MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 1 OF 28

MFT 701

Fundamentals of Marriage and Family Therapy

14 teaching weeks · 10 practicals · 8 worked cases · 7 figures

SEMESTER ONE

1.71 units · Prerequisite: none

> “This is the course that turns a helping professional into a systemic one.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 701 — Fundamentals of Marriage and Family Therapy

MSc Marriage and Family Therapy · Semester One · 1.71 Units · 14 teaching weeks · Prerequisite: none

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	**Fundamentals of Marriage and Family Therapy**	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is the course that turns a helping professional into a systemic one. It covers the intellectual history of family therapy, the epistemology that separates it from individual work, the profession's identity and scope, and the basic architecture of a first session. Everything later in the programme — theory, technique, assessment, ethics — assumes the vocabulary built here.

How to use this book

Each week contains four things: **the idea** (what you must understand), **the practice** (what you must be able to do), **the case** (what it looks like in a room), and **a reflective task** (what it asks of you personally). Read the week before the lecture; do the practical in the lab; write the reflection within 48 hours, while it is still uncomfortable.

The book is written to be read straight through, but it is also designed for revision: the integration chapter at the end compresses the whole course into two pages, and the glossary defines every term the examination may use.

1. Course description and rationale

Most mental health training teaches us to look inside a person: their thoughts, their symptoms, their attachment history. Family therapy teaches us to look between people: the loop, the triangle, the rule that nobody wrote down. This is not a rejection of individual psychology. It is an additional discipline of attention, and it changes what counts as the problem, the cause, and the cure.

Consider three common presentations:

Presentation	Individual formulation	Systemic formulation
A 15-year-old refuses school	Anxiety disorder, school phobia	A child whose symptoms hold a parental conflict at bay; the school refusal is the family's most effective way of preventing an argument about the marriage
A wife is "depressed"	Major depressive episode	A woman in a triad with her husband and his mother, carrying the family's unspoken grief about a lost pregnancy
A man "works too much"	Workaholism, avoidance	The person who regulates the family's emotional temperature by being absent, in a system that rewards his absence

None of these systemic formulations is automatically more true. Each is a hypothesis with different implications for where you intervene. The mature clinician holds both — the individual and the relational — and chooses deliberately.

Why this course first. Because students who begin with technique learn to do something they cannot explain, and students who begin with theory learn to explain something they cannot do. MFT 701 begins with framing: what kind of problem is this, and therefore what kind of help is required?

2. Learning outcomes

By the end of the course, students will be able to:

#	Outcome	Assessed in
LO1	Explain the epistemological shift from linear to circular causality, and its clinical consequences	Exam, Week 2 log
LO2	Trace the historical development of family therapy and locate contemporary models within it	Exam
LO3	Describe the profession's identity, scope of practice, and relationship to allied professions	Assignment
LO4	Conduct a structured first session, including joining, problem definition, and contracting	Skills assessment
LO5	Draw a three-generation genogram and generate hypotheses from it	Assignment
LO6	Critically discuss the common-factors evidence and its implications for model choice	Exam essay
LO7	Identify their own family-of-origin patterns and describe their effect on clinical work	Reflective log

3. Assessment

Component	Weight	Due
Reflective practice log (weekly, 300–400 words)	20%	Fortnightly
Genogram and hypothesis assignment (2,000 words + diagram)	30%	Week 9
Two-hour written examination (seen case + essay + short answers)	50%	Week 15

Skills gate. Students must pass a 15-minute observed first session (Week 6) before proceeding to MFT 706. The gate is pass/fail with one resit.

4. The fourteen-week teaching plan

Week	Topic	Practical	Core reading
1	What is family therapy? The relational turn	Triad: "the problem as described"	Nichols ch. 1; Handbook ch. 1
2	Linear and circular causality; first- and second-order cybernetics	Observation of a recorded argument	Watzlawick, Pragmatics ch. 1
3	A short history: from schizophrenia research to evidence-based practice	Timeline construction	Nichols ch. 2–3
4	Systems thinking: boundaries, feedback, homeostasis, rules	Feedback-loop mapping exercise	Becvar & Becvar ch. 2

Week	Topic	Practical	Core reading
5	The profession: identity, scope, settings, teamwork	Role boundaries case discussion	AAMFT code, scope sections
6	The first session I: joining, and the referral question	**Observed skills gate**	Minuchin & Fishman ch. 1
7	The first session II: defining the problem in the family's language	Triad: problem definition	Haley, Leaving Home ch. 1
8	The genogram as a hypothesis machine	Genogram lab (own family)	McGoldrick, Genograms ch. 1–2
9	Formulating relationally: the 5-P grid applied to a family	Formulation workshop	Dallos & Draper ch. 4
10	Culture, class and power in the consulting room	Reflexivity exercise	McGoldrick, Ethnicity & Family Therapy
11	Evidence: what works, for whom, and the common factors	Appraisal of one RCT and one qualitative study	Wampold ch. 2
12	Ethics, risk and the limits of the systemic frame	Risk-screen rehearsal	MFT 704 pre-reading
13	Integration I: three models applied to one family	Live demonstration + discussion	Nichols ch. 10
14	Integration II: your emerging clinical stance	Position statement presentation	Course review

Week 1 — What is family therapy? The relational turn

The idea. Family therapy is a frame, not a set of techniques. It is the decision to treat the relationship as the client. That decision has four consequences:

1. **The unit of assessment changes.** You must hear from more than one person, and notice what happens between them.
2. **The unit of intervention changes.** You intervene in sequences, rules, meanings and structures, not only in individual cognition.
3. **The definition of success changes.** Success may be a family that argues well, not a family that never argues.
4. **The therapist's position changes.** You are not a technician outside the system; you are a participant who affects it (Figure 1).

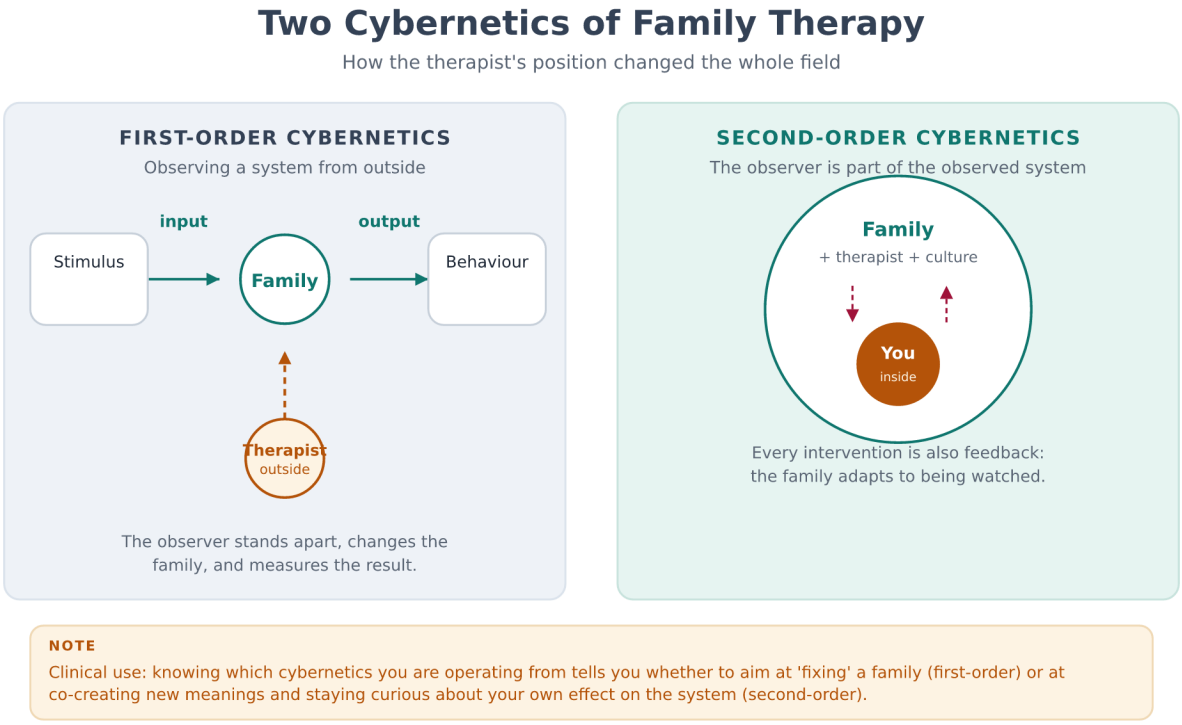


Figure 1.1 — First- and second-order cybernetics: where the therapist stands

A working definition. Marriage and family therapy is the assessment and treatment of relational systems — couples, families, and the wider networks in which they live — using interventions that target interaction patterns, meaning systems, and the emotional bonds between people.

What it is not. It is not "seeing everyone together" as a matter of policy. Sometimes the right systemic move is an individual session within family work. It is not a belief that individuals do not exist, or that suffering is imaginary. It is a disciplined focus on context.

> **CASE**
> Mrs Okonkwo brings her 9-year-old son, Ebuka, for "stubbornness". In the first ten minutes she mentions, twice, that his father "is never home". The linear frame says: assess and treat Ebuka's behaviour. The systemic frame says: ask who else is affected by the behaviour, what happens after it, and what would change in the household if it stopped. The second question is not a rejection of the first; it is a widening of the field in which the first can be treated.

Practice (lab). In triads, one person describes a current difficulty for three minutes. The "therapist" may only ask questions that include another person ("who notices when…", "what does your mother do when…"). Debrief: what did the widening do to the description?

> **REFLECT**
> Write down the last significant conflict in your own family. Now write it again from the point of view of the person you understood least. Note what changed in your body as you wrote.

Week 2 — Linear and circular causality

The idea. Linear causality asks who started it and what caused it. Circular causality asks what maintains it and how each response feeds the next. This is the single most important conceptual shift in the course.

The shift is not merely philosophical. It changes what you do:

Linear question	Circular question
"When did the depression begin?"	"What happens in the house when she stays in bed?"
"Why does he lose his temper?"	"What does she do just before he raises his voice?"
"Who is to blame?"	"How does the blaming itself work?"

Punctuation. Each family member punctuates a sequence differently — she says he withdraws, he says she nags — and each believes the other started it. The therapist's job is not to decide who is right but to make the sequence visible so that both can see the same film (Figure 2).

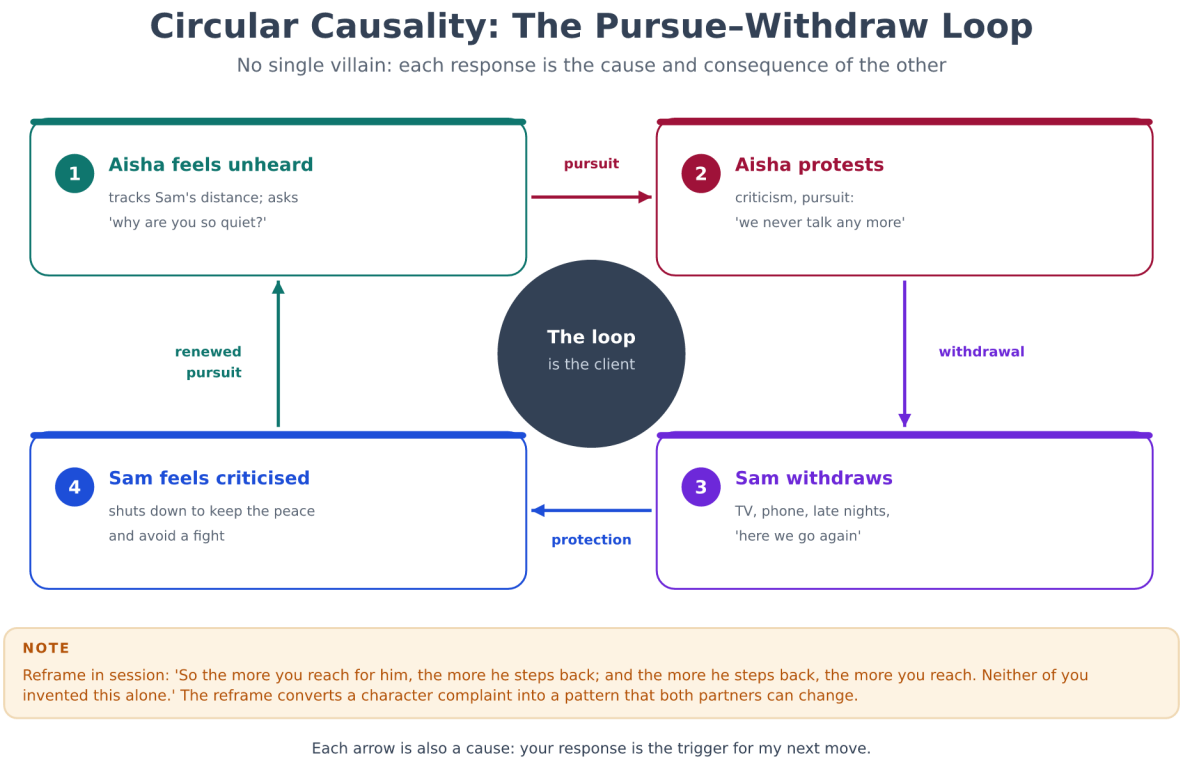


Figure 2.1 — The pursue–withdraw loop: a circular formulation

First-order and second-order cybernetics. First-order cybernetics treats the observer as outside the system: you measure, you intervene, you evaluate the result. Second-order cybernetics accepts that the observer is part of the system being observed: the family responds to being watched, and your presence is data. Contemporary practice is mostly second-order, which is why reflexivity (Week 10) is a clinical skill rather than a personality trait.

> **CASE**
> In a couple session, Folake says Dele "starts every fight". Dele says Folake "is never satisfied". Rather than adjudicate, the therapist asks each to describe, minute by minute, what happened last Tuesday. The sequence that emerges is: Folake raises a worry → Dele answers with a joke → Folake escalates → Dele leaves the room → Folake follows him → Dele goes silent for two days. Nobody started it. The loop started it.

Practice (lab). Watch five minutes of a recorded (consented) or scripted argument with the sound off. Write the sequence as a loop of four to six steps, with an arrow for each move. Then watch with sound and compare your loop with what the speakers said caused it.

> **KEY**
> If you can draw the loop, you can intervene in it. If you can only name a villain, you are still thinking linearly.

Week 3 — A short history of the field

You cannot critique a tradition you cannot narrate. This week gives you the story in five movements.

1. The pre-history (1900s–1950s). Child guidance clinics, marriage counselling, and social casework treated families, but individually. Psychoanalysis dominated, and the family was understood as the source of individual pathology rather than as a system in its own right.

2. The research rupture (1950s–1960s). Several independent groups — Bateson's Palo Alto project, Lidz and Wynne studying families of people diagnosed with schizophrenia, Bowen at Menninger, Ackerman in New York — began observing whole families and describing patterns rather than causes. Bateson's team gave us the double bind hypothesis and, more durably, the vocabulary of communication and paradox.

3. The heroic era (1960s–1970s). Charismatic founders built schools: Minuchin (structural), Haley and Madanes (strategic), Satir (experiential/human validation), Whitaker (symbolic-experiential), Bowen (multigenerational), Boszormenyi-Nagy (contextual), the Milan group (systemic/milan). Techniques were bold, sessions were long, and therapists were often filmed behind one-way mirrors.

4. The critique and the turn to meaning (1980s–1990s). Feminists (Hare-Mustin, Goldner, Walters) demonstrated that "neutral" systemic descriptions hid gendered power. Social constructionism and postmodernism (Anderson & Goolishian, White & Epston) shifted attention from structure to story. The Milan team's later work introduced curiosity and the reflecting team.

5. Evidence and integration (1990s–present). Manualised models were tested in randomised trials (behavioural couples therapy, EFT, multisystemic therapy, functional family therapy, multidimensional family therapy). Attachment, neuroscience and trauma research entered the field. The current mainstream is integrative: model-informed, evidence-aware, culturally situated.

Why the history matters clinically. Every school was a reaction to a limitation of its predecessor. Structural therapy reacted to an over-focus on insight; narrative therapy reacted to an over-focus on expertise; trauma-informed care reacted to an over-focus on technique. When you find yourself irritated by a model, ask which limitation it is answering — you will usually find your own blind spot there.

PRACTICAL

Build a one-page timeline in small groups. For each decade, write (a) the dominant idea, (b) the model it produced, (c) the critique that ended its dominance. Present one decade each.

Week 4 — Systems concepts you will actually use

Six concepts do most of the clinical work. Learn them precisely; vagueness here produces vague therapy.

1. System and subsystem. A system is a set of elements in interaction such that a change in one produces a change in the others. Subsystems (the couple, the siblings, the parental dyad) are organised by boundaries.

2. Boundary. The invisible rules about who participates and how. Boundaries range from diffuse (enmeshed: little privacy, feelings shared before they are owned) to rigid (disengaged: autonomy without belonging). Healthy is not the middle of a line; healthy is appropriate to the task — the parental subsystem needs a clearer boundary than the sibling subsystem (Figure 5 in MFT 702).

3. Feedback. Negative feedback corrects deviation (a thermostat, a bedtime routine); positive feedback amplifies it (an escalation, a panic spiral). Families contain both, always. Symptoms often persist because a negative-feedback loop protects a positive-feedback loop underneath.

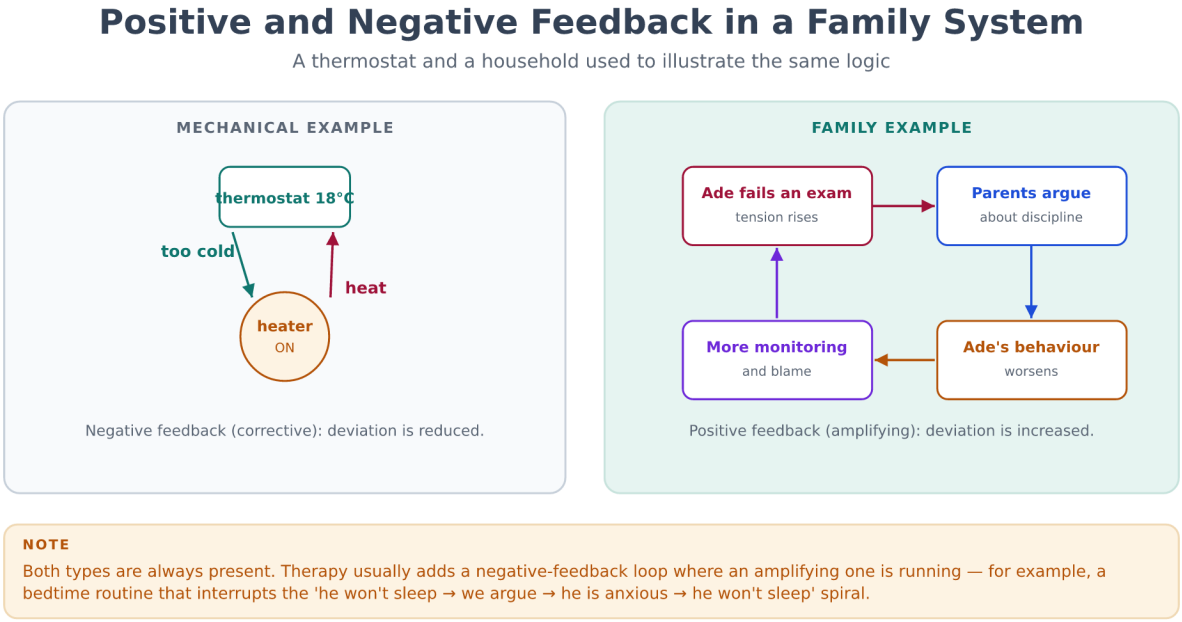


Figure 4.1 — The same feedback logic in a thermostat and in a family

4. Homeostasis. The system's tendency to maintain its familiar organisation, including familiar pathology. This is why a family may resist improvement: the drinking, the fighting, or the symptom has become load-bearing.

5. Rules. Recurrent patterns that prescribe and proscribe behaviour ("we do not discuss money"; "no one is allowed to be angry at Father"). Most family rules are never spoken; they are inferred from what produces anxiety.

6. Triangles. When tension between two people exceeds what the dyad can hold, a third is recruited — a child, an affair, a job, an in-law, a symptom. Triangles distribute anxiety; they also freeze the dyad's capacity to resolve it (covered fully in MFT 715).

> **CASE**
> Since the father's redundancy, the household has run a nightly cycle: he criticises the children's homework, the mother defends them, he accuses her of undermining him, she goes to bed early, and the eldest daughter stays up to calm him. Mapping it: the father's shame (unspoken) → criticism → mother's protection → his isolation → daughter's role as regulator. Nothing here is "the problem". The system has found a way to hold his shame — at the cost of a 14-year-old's adolescence.

Practice (lab). Using the case above, label each element: boundary, feedback type, homeostatic function, rule, triangle. Then answer: what would have to change elsewhere if the daughter stopped regulating?

Week 5 — The profession: identity, scope and settings

Family therapy is a distinct profession with its own training standards, codes and legal standing, and it is also a transdisciplinary competence. You need both facts.

Scope of practice. Assessing and treating relational distress; couple and marital therapy; family therapy with children and adolescents; parent guidance; separation and divorce work; work with stepfamilies; group work with relational focus; consultation to organisations. In most jurisdictions, family therapists do **not** prescribe medication, conduct forensic custody evaluations of their own clients, or give legal advice.

Settings. Private practice, community mental health, hospitals (including paediatric and psychiatric liaison), schools, child protection services, addiction services, employee assistance programmes, faith-based counselling centres, NGOs in humanitarian settings, and court-connected mediation.

Teamwork. You will work alongside psychiatrists, paediatricians, social workers, teachers, probation officers, imams and pastors, and community health workers. Two skills matter more than you expect: writing a referral that says what you want, and knowing the boundaries of what you may disclose.

Professional identity. Three tensions recur in the literature and in practice:

1. **Expert or collaborator?** The field has moved from therapist-as-director to therapist-as-curious- participant, but families often want expertise. The mature position is "expert in process, not in their lives".
2. **Neutral or advocating?** Systemic neutrality does not require moral neutrality about violence.
3. **Specialist or generalist?** You will be asked to work with issues you were not trained for. The professional answer is competence boundaries plus referral, not improvisation.

> **CASE**
> A school refers a 12-year-old for "aggression". The school wants an assessment letter; the mother wants the school to stop the bullying; the father does not want to attend. Assume nothing. Ask who defined the problem, who benefits from that definition, and what the referral is for. Then write down your scope: what you will assess, what you will not, and to whom you will send what.

> **PRACTICAL**
> Draft a one-page "Scope of practice" statement for yourself as an intern. Include the three things you will not do without supervision.

Week 6 — The first session I: joining

The idea. Joining is the process of becoming part of the system sufficiently to influence it, without becoming so much a part of it that you lose the capacity to observe. Minuchin called it the therapist's "accommodation" to the family's style. It is not friendliness; it is deliberate.

Four joining moves:

1. **Track the family's language.** Use their words, not your diagnostic ones. If they say "he is stubborn", ask about "stubborn" before you ask about "oppositionality".
2. **Validate what is true in each person's account.** Not "you are right", but "that makes sense given what you have been carrying".
3. **Match pace and formality.** Some families want a structured interview; others need ten minutes of narrative before any structure is tolerable. Ask: "How would you like to use this hour?"
4. **Make the structure explicit.** Who is here, why, for how long, who knows, what is confidential. Anxiety drops when the frame is clear.

What to cover in a first session (the "minimum viable first session"):

Element	Sample wording
Contract and limits	"I will keep what is said here private, except where someone is at risk of harm."

Element	Sample wording
Each person's view	"I would like to hear from each of you — what worries you most?"
The attempted solutions	"What have you already tried? What happened?"
The pattern	"So when X happens, what does Y do? And then?"
Risk screen	"Is anyone being hurt at home? Any thoughts of ending your life?"
Feedback	"Here is what I think I heard. What have I got wrong?"
Next step	"I would like to see all of you again next week, including your father."

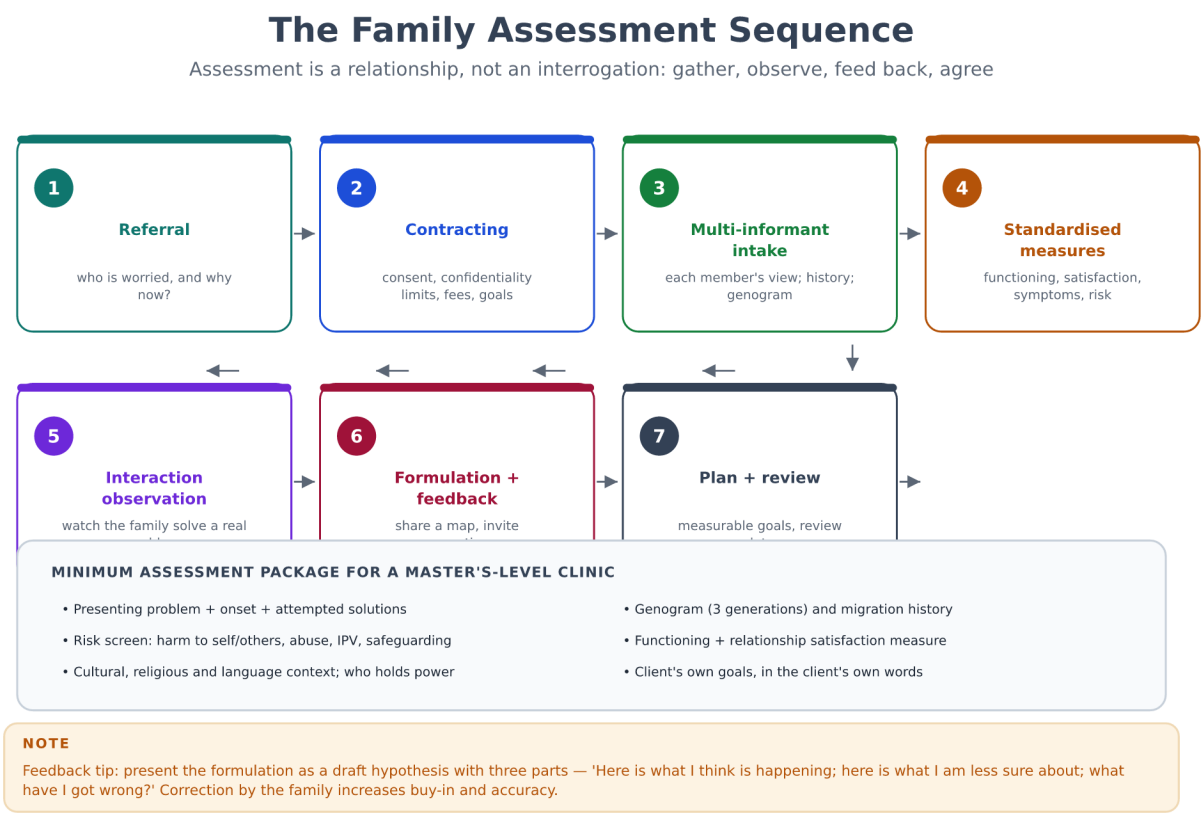


Figure 6.1 — The assessment sequence: assessment is a relationship, not an interrogation

CASE

A family arrives furious with each other and with the referrer. The therapist's first sentence is not a question but an acknowledgment: "You have been sent here by three different people and none of them asked you what you wanted. Let's start there." Joining succeeded before any information was gathered.

PRACTICAL (SKILLS GATE)

Fifteen minutes, observed. You must (1) state confidentiality and its limits, (2) obtain a description of the problem from at least two people, (3) ask one circular question, (4) complete a risk screen, (5) offer a summary and invite correction. You may use one index card of prompts. Feedback uses the four-point structured sheet in the programme handbook.

Week 7 — The first session II: defining the problem

The referral question is rarely the clinical question. Your task is to negotiate a problem definition that is (a) specific, (b) interactional, (c) owned by the family, and (d) changeable.

From vague to specific. "He is disrespectful" → "When I ask him to help, he looks at his phone and says nothing. That happens most evenings, and then I shout."

From individual to interactional. "She is manipulative" → "When she cries, her father gives in and you feel undermined; then you withdraw, and she comes to me." Notice: no blame is removed, but the description now includes three people and a sequence.

From imposed to owned. Ask each person to complete: "I will know this has worked when…". Write the answers down and read them back. If the parents' answers are incompatible, that incompatibility is the work.

From unchangeable to changeable. "I want him to respect me" is a relational outcome that no one can deliver on demand. "I want us to agree on one consequence for phone use, and both apply it for a week" is testable.

CASE

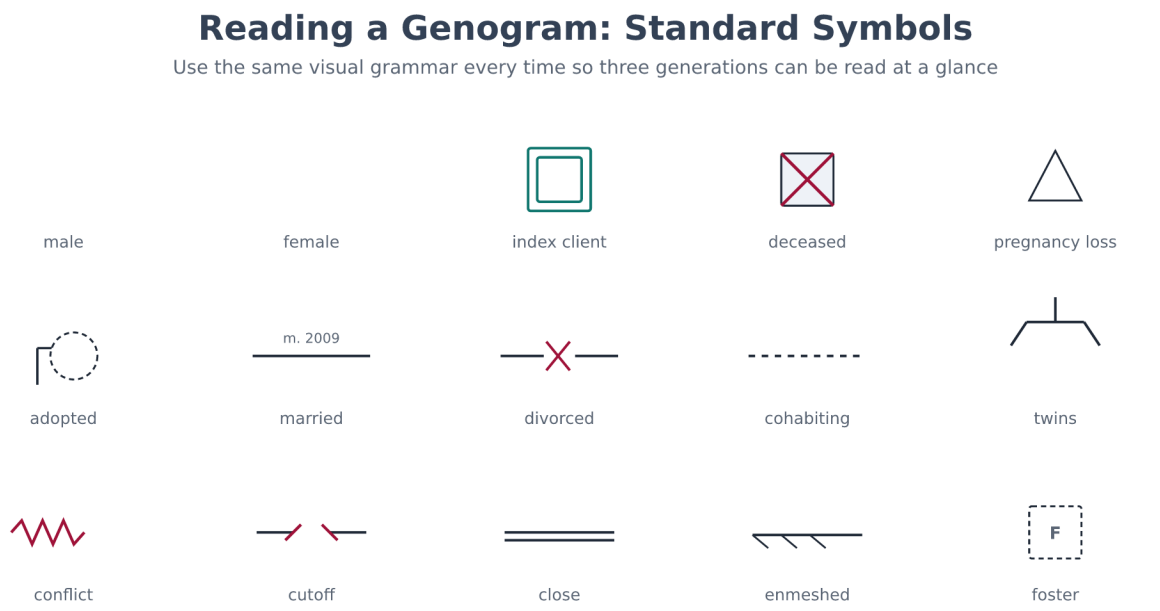
The Adeyemis come because their 16-year-old, Amara, has refused school for six weeks. The referral says "school refusal". The negotiated definition becomes: "The mornings have become a battle that everyone loses; Amara does not leave the house; her mother stays home from work; her father works later; and her brother has started sleeping in her room." The problem is now a system, not a girl.

Practice (lab). Take a complaint a peer brings from placement (disguised) and rewrite it four times: specific, interactional, owned, changeable. Read the final version to the "family" and ask whether it is recognisable.

Week 8 — The genogram as a hypothesis machine

The genogram is a three-generation map of family structure, relationships and events. Drawing it is simple; using it is not — the value lies in the hypotheses it generates and the questions it permits.

Standard symbols (Figure 3) and conventions: squares for males, circles for females, horizontal line for marriage, slash for divorce, cross for death, double outline for the index client. Relationship lines: double for close, zigzag for conflict, broken with slashes for cutoff, triple for enmeshed.



NOTE

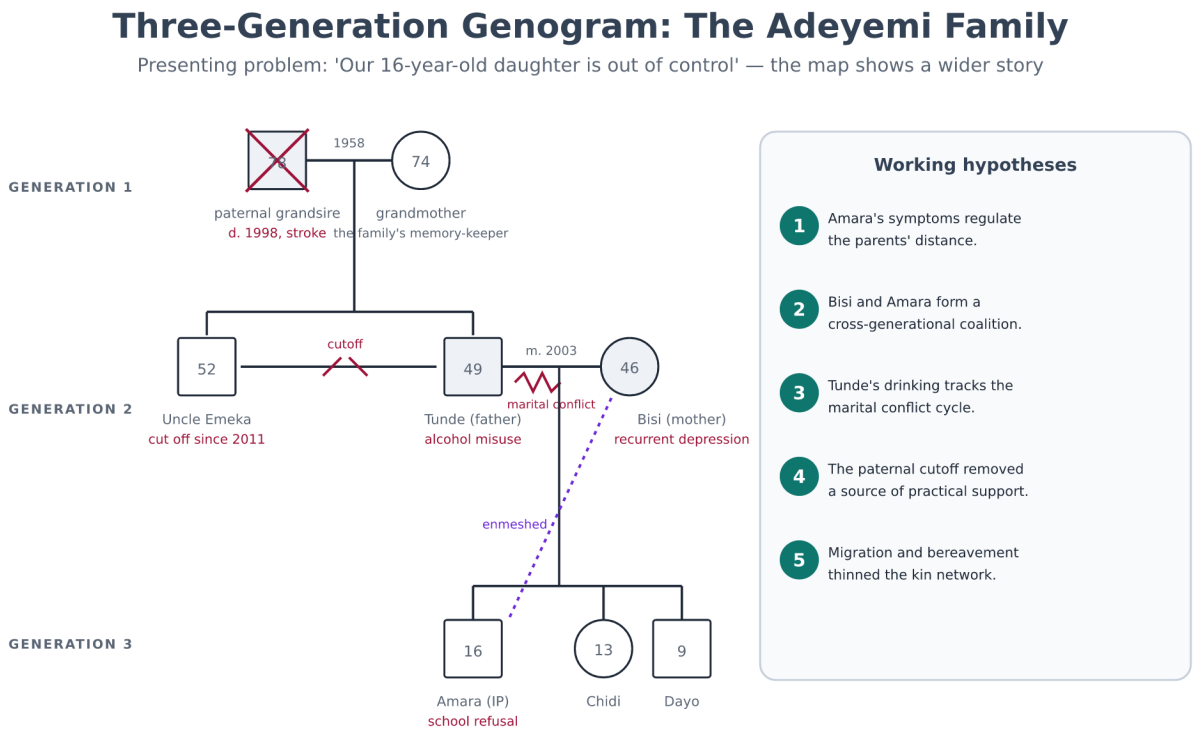
Also record: ages or year of birth inside the symbol, date and cause of death above it, occupation, migration, religion, and significant secrets (affairs, incarceration, abuse) using a key that you explain to the family before you draw.

Figure 3 — Genogram symbols: learn the grammar once and three generations become readable

What to ask while drawing. Age and year of birth; occupation and migration; deaths and their causes; separations, divorces, remarriages; who is close to whom; who does not speak to whom; who is the "responsible one"; who "had a breakdown"; family secrets; repeating names; repeating ages at which things happened.

From map to hypothesis. A good genogram yields three kinds of hypothesis:

1. **Structural** — coalitions, cutoffs, over/under-functioning pairs.
2. **Temporal** — patterns repeating at the same age, or symptom onset clustering around anniversaries.
3. **Legacy** — loyalty, indebtedness, roles inherited ("the eldest daughter always cares for everyone").



NOTE

Notice what the map does NOT tell you: the meaning the family gives to father's drinking, the voice of the absent uncle, and the resources (church, neighbours, income) that are not drawn. A genogram is a hypothesis generator, not a verdict.

Figure 8.1 — A worked three-generation genogram with hypotheses

Cultural note. In many families the genogram must include people who are not biologically related but are structurally central — a foster child, a co-wife, a pastor, a long-term house help. Ask, "who else belongs to this family?" before you decide the map is complete.

PRACTICAL

Draw your own three-generation genogram from memory first, then correct it with a relative. Note every place where your memory and theirs differ. Those differences are the assignment.

Week 9 — Formulating relationally

A formulation is a provisional, testable explanation that connects the presenting problem to the system that maintains it, and points to where you would intervene.

Use the 5-P grid (presenting, predisposing, precipitating, perpetuating, protective) and add two relational columns: who is involved and what function the symptom serves.

Domain	Adeyemi family
Presenting	Amara, 16, school refusal × 6 weeks; parental conflict over discipline
Predisposing	Maternal depression; paternal alcohol misuse; migration severed the extended-family buffer
Precipitating	Father's redundancy; best friend left the city; transition to senior secondary
Perpetuating	Pursue–withdraw cycle between parents; mother–daughter coalition; avoidance rewarded by anxiety relief; disrupted sleep
Protective	Both parents attend; church community; Amara's engagement with art; an aunt who is trusted

The intervention sentence. End every formulation with one sentence of the form: "Given X, Y and Z, the most useful focus now is W, because it is the most changeable and the most load-bearing." For the Adeyemis: "Given the coalition and the parental distance, the most useful focus now is the couple's conflict cycle, because it is what the symptom is currently regulating."

Share it. A formulation that is never spoken is a private theory. Offer it as a draft with three parts: here is what I think, here is what I am unsure about, here is what I need from you.

> **ASSIGNMENT (30%)**
> 2,000 words plus a genogram. Choose a disguised or composite family from placement or a detailed role-play. Include: genogram, 5-P grid, circular formulation with a diagram, three competing hypotheses, one intervention plan with rationale, and a reflection on your own reactivity to this family.

Week 10 — Culture, class and power

Every family you meet is embedded in a culture, and so are you. Culture is not a variable to be "controlled for"; it is the medium in which the problem is defined.

Four practical disciplines.

1. **Ask rather than assume.** "In your family, who usually makes the big decisions? Who is consulted? Is that how you want it to be?"
2. **Locate yourself.** Complete the same cultural genogram for your own family. Your views on hierarchy, individuation, duty and disclosure are not neutral.
3. **Notice the power in the room** — including yours. The family may be polite because of your status, not because they agree.
4. **Interrogate "normal".** Models that prize individuation can pathologise families who prize interdependence. Ask what the family's own picture of a good life is.

Language. If the family thinks in their first language about emotional matters, the session may need that language or an interpreter. Interpreters are not furniture: brief

them, seat them beside you, ask them to interpret speech not summaries, and never ask them to translate a disclosure you have not prepared for.

Religion and spirituality. In much of the world, faith is not an "aspect" of family life; it is the organising frame. Ask what the family's faith says about the problem, who in the religious community knows, and whether they want that person involved. Do not compete with the pastor or imam; coordinate.

> **CASE**
> A couple is referred after the wife disclosed to her pastor that her husband has been violent twice. She does not want the police involved and does not want her family told. The therapist must hold three things at once: respect for her autonomy and faith, a safety plan that does not depend on him changing, and honesty about the limits of confidentiality. (Full treatment in MFT 712 and 724.)

> **REFLECT**
> Where in your own family would a therapist from a different culture misread the structure? Write 300 words on what they would get wrong and why.

Week 11 — What the evidence says

Students often arrive believing either that therapy "just works" or that only manualised treatments do. The evidence is more interesting than either position.

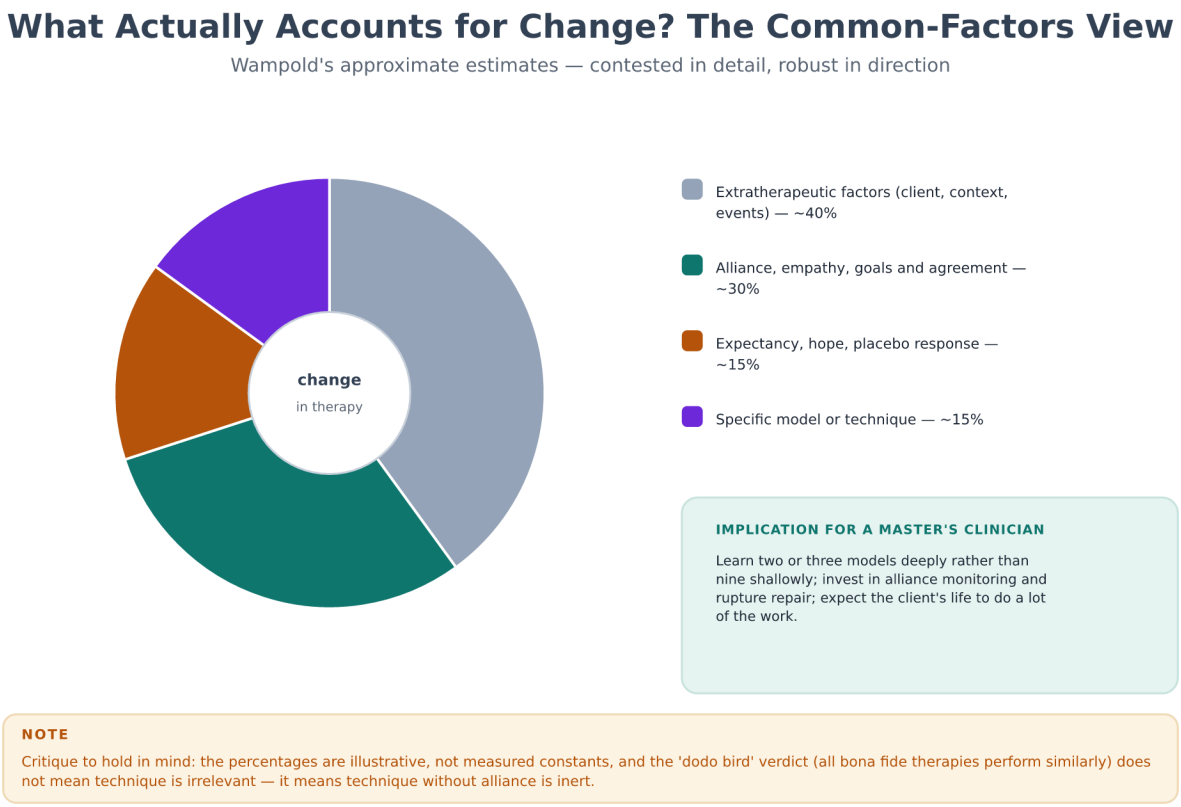


Figure 11.1 — Common factors and model effects, with a critique

Four findings worth carrying into practice.

1. **Family and couple therapies work.** Meta-analyses consistently find moderate-to-large effects for couple therapy in relationship distress, and for family-based treatments in adolescent behaviour problems, eating disorders (family-based treatment for anorexia is a first-line intervention), substance misuse and psychosis relapse.
2. **Alliance matters, and it is relational.** In couple and family work, the alliance is not one relationship but several, and shared sense of purpose predicts outcome better than any single dyad's alliance with the therapist.
3. **Model differences are smaller than model loyalties suggest.** Common-factors research attributes much of the variance to client factors, alliance and expectancy. Technique is not irrelevant: it organises what the therapist notices and does, and it matters more for specific problems.
4. **Drop-out is the silent failure.** In couple and family work, 20–40% of families end therapy unilaterally. Attendance, engagement and early change are the best predictors of retention. Track them.

How to read a paper in ten minutes. Read the abstract; then the design (who, how many, compared to what); then the outcome measure (does it measure what you care about?); then the effect size, not the p-value; then the limitations; then ask whether the sample resembles your clinic.

> **CRITICAL READING**
> Choose one randomised trial of a family therapy model and one qualitative study of family therapy experience. Write 500 words comparing what each can and cannot tell you.

Week 12 — Ethics, risk and the limits of the systemic frame

Ethics is not a chapter at the end of the course; it is the frame that makes the work possible. Here we establish the four issues every family therapist meets in the first month.

1. Confidentiality in a multiperson system. There is no single confidentiality. Agree the rules at the start: what is said in the joint session is shared; what is told to you individually may need to be brought into the open; secrets that affect safety cannot be kept. Write the policy down and give it to the family.

2. Risk. Screen for suicide, self-harm, violence, abuse and neglect in every new case, and document it. You cannot assume a colleague has done it.

3. Competence. Taking a case you are not trained for is not courage. It is a risk to the client and to you.

4. Multiple relationships and power. Small communities, religious networks and family ties make dual relationships likely. The test is not "is there a second relationship?" but "does the second relationship impair the first, or exploit the first?"

> **CASE**
> During an individual session, a husband tells you he has resumed an affair and asks you not to tell his wife. He is not at risk and no one is being harmed in the sense that requires disclosure. What do you do? (Work this case in groups; then compare with the decision model in Figure 33, MFT 704.)

Documentation. Write notes as if a colleague will take over tomorrow and a court may read them in a year: date, attendees, presenting issues, risk screen result, interventions, plan, and who was told what.

Week 13 — Integration I: three lenses on one family

This week is a rehearsal for MFT 702. You will apply three models to the same case and notice that each produces a different question, a different focus and a different first move.

Model	Key question	First move with the Adeyemis	Likely risk
Structural	What is the hierarchy, and where are the boundaries?	Strengthen the parental subsystem; block the mother–daughter coalition	Imposing a structure the family experiences as foreign

Model	Key question	First move with the Adeyemis	Likely risk
Bowen/multigenerational	How does anxiety travel, and who absorbs it?	Coach the parents to manage their own reactivity; map the projection onto Amara	Slow; family may want symptom relief now
Solution-focused	What is already working, and what would "one point better" look like?	Find exceptions to school refusal; scale the parents' confidence	May bypass the function of the symptom

The point of the exercise is not to choose a winner. It is to notice that a family presents a plural reality and that your model determines what you can see. Good clinicians can move between at least two frames without losing coherence.

> **PRACTICAL**
> In groups, take the same case and produce three one-page plans. Present all three. Then answer: which would you start with, and what would make you change your mind?

Week 14 — Integration II: your emerging clinical stance

Write a two-page **position statement**: what you believe about how relational problems develop, what you think therapy does, what you will not do, and what you still do not know. This is not an exam answer; it is the beginning of a professional identity that will be revised for the rest of your career.

A strong statement includes: one theoretical commitment, one epistemological commitment (how you know what you know), one ethical commitment, one cultural commitment, and one honest limitation.

> **REFLECT**
> Read your Week 1 reflection again. What has changed? What has not? Write 300 words to yourself, dated, to be reread at the end of the programme.

Integration: the whole course on two pages

1. The frame. Treat the relationship as the client. Widen the field; include context; notice the observer.

2. Causality. Move from who-started-it to what-maintains-it. Draw the loop. Punctuate differently.

3. History. Five movements: pre-history, research rupture, heroic era, critique and meaning, evidence and integration. Each school answers a limitation of its predecessor.

4. Concepts. System, subsystem, boundary, feedback, homeostasis, rule, triangle. Six concepts, most of the clinical work.

5. Profession. Know your scope, your setting, your team, your limits. Refer well.

6. The first session. Join, contract, hear everyone, screen for risk, map the pattern, summarise, invite correction, agree next step.

7. The problem definition. Specific, interactional, owned, changeable.

8. The genogram. Three generations, structural and temporal hypotheses, cultural completeness.

9. Formulation. 5-P plus "who is involved" and "what function the symptom serves", ending with one intervention sentence.

10. Culture and power. Ask, locate yourself, notice power, interrogate normal.

11. Evidence. Family therapy works; alliance matters; models overlap; drop-out is the silent failure.

12. Ethics. Confidentiality is negotiated, risk is screened, competence is bounded, dual roles are examined, notes are written for strangers.

13. Integration. Multiple lenses; move between frames without losing coherence.

14. Stance. A written, revisable position on what you believe and what you do not know.

Glossary

Boundary — the implicit rules governing participation and closeness between subsystems. **Circular causality** — an account in which each element both causes and is caused by the others. **Coalition** — an alliance of two or more people against a third. **Cybernetics** — the study of control and communication in systems; first-order (observer outside), second-order (observer inside). **Detriangulation** — the process of extricating a third party from a dyadic conflict. **Enmeshment** — diffuse boundaries with excessive emotional sharing and low autonomy. **Disengagement** — rigid boundaries with little emotional contact. **Feedback (negative)** — a process that reduces deviation and restores stability. **Feedback (positive)** — a process that amplifies deviation and produces escalation. **Genogram** — a three-generation diagram of family structure, relationships and events. **Homeostasis** — the system's tendency to maintain its existing organisation. **Hypothesis** — a provisional, testable explanation that guides enquiry. **Joining** — the therapist's deliberate accommodation to a family's style in order to work with it. **Linear causality** — an account in which A causes B in one direction. **Punctuation** — the way each participant marks the beginning of a sequence, thereby assigning blame. **Reframe** — relabelling a behaviour so that it is understood in a new, usually relational, frame. **Subsystem** — a unit within a system organised by boundaries (couple, siblings, parental dyad). **System** — a set of interacting elements such that a change in one changes the others. **Triangle** — a three-person configuration that absorbs and stabilises dyadic tension.

Self-assessment

Multiple choice (answers below)

1. A therapist says, "The real question is not who started the argument but what keeps it going." This statement reflects: (a) linear causality (b) circular causality (c) psychodynamic causality (d) moral causality

2. A father's drinking has the effect of preventing his wife and son from discussing the son's expulsion. In systemic terms the drinking is: (a) random (b) homeostatic (c) an individual symptom only (d) irrelevant to the family

3. "We never discuss money" is best described as: (a) a boundary (b) a family rule (c) a triangle (d) a reframe

4. In second-order cybernetics the therapist is: (a) outside the system (b) part of the observed system (c) a neutral observer (d) irrelevant

5. The best first move with a family that arrives furious at the referrer is: (a) correct the misunderstanding immediately (b) acknowledge their experience of being sent (c) begin the assessment (d) ask who is the identified patient

6. A negotiated problem definition should be specific, interactional, owned and: (a) clinically imposed (b) changeable (c) blame-allocating (d) historical

7. Which is not an element of a minimum first session? (a) confidentiality and limits (b) a risk screen (c) a full genogram (d) a summary and invitation to correct

8. The strongest predictor of drop-out in couple and family work is: (a) diagnosis (b) therapist's model (c) early engagement and early change (d) family size

9. "Multigenerational transmission" belongs to which tradition? (a) structural (b) Bowen (c) Milan (d) solution-focused

10. The statement "expert in process, not in their lives" describes: (a) neutrality about violence (b) the therapist's stance (c) a legal requirement (d) a diagnostic rule

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 c · 8 c · 9 b · 10 b

Short answer (150 words each)

1. Explain the difference between negative and positive feedback using a family example.
2. Why is "punctuation" clinically useful? Give a sample question.
3. Name three limits of confidentiality in family work and how you would state each to a family.
4. What is the difference between a genogram and a formulation?
5. Summarise the common-factors critique of model loyalty in four sentences.

Essay questions (choose one, 1,500 words)

1. "Family therapy is a frame, not a technique." Discuss with reference to at least two models and one case from your placement.
2. Critically evaluate the claim that systemic neutrality is impossible — and undesirable — in cases of domestic violence.
3. Using a disguised case, demonstrate how a linear formulation and a circular formulation would lead to different treatment plans. Which would you choose and why?

Practical and clinical log requirements

For each session you observe or conduct, record:

Field	Requirement
Date and duration	Exact
Attendees	Relationship to index client; ages; who was absent
Referral source and question	In the referrer's words
Presenting problem	In each attendee's words
Risk screen	Completed Y/N; result; action
Pattern observed	One loop or sequence, described behaviourally
Intervention	What you did, and why
Response	What happened next
Supervision	Who, when, what was discussed
Plan	Next step, with review date

The log is assessed for **specificity and consistency**, not for how impressive the case was. A log of ordinary sessions written precisely will score higher than a dramatic case written vaguely.

Further reading

Core texts

- Nichols, M. P., & Davis, S. (2021). Family Therapy: Concepts and Methods (12th ed.). Pearson.
- Becvar, D. S., & Becvar, R. J. (2018). Family Therapy: A Systemic Integration (9th ed.). Pearson.
- Dallos, R., & Draper, R. (2015). An Introduction to Family Therapy (4th ed.). Open University Press.

Foundations and history

- Watzlawick, P., Beavin, J., & Jackson, D. (1967). Pragmatics of Human Communication. Norton.
- Bateson, G. (1972). Steps to an Ecology of Mind. Chandler.
- Hoffman, L. (1981). Foundations of Family Therapy. Basic Books.
- Gurman, A. S., Lebow, J. L., & Snyder, D. K. (Eds.). (2015). Clinical Handbook of Couple Therapy (5th ed.). Guilford.

Practice and technique

- Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press.
- Haley, J. (1980). Leaving Home. McGraw-Hill.
- McGoldrick, M., Gerson, R., & Petry, S. (2020). Genograms: Assessment and Treatment (4th ed.). Norton.

Culture and critique

- McGoldrick, M., Giordano, J., & Garcia-Preto, N. (Eds.). (2005). Ethnicity and Family Therapy (3rd ed.). Guilford.
- Hare-Mustin, R. (1978). A feminist approach to family therapy. Family Process, 17(2), 181–194.

Evidence

- Wampold, B. E., & Imel, Z. E. (2015). The Great Psychotherapy Debate (2nd ed.). Routledge.
- Sprenkle, D. H., Davis, S. D., & Lebow, J. L. (2009). Common Factors in Couple and Family Therapy. Guilford.
- Shadish, W. R., & Baldwin, S. A. (2003). Effects of family and marital psychotherapies: A meta-analysis. Journal of Consulting and Clinical Psychology, 71(6), 1080–1097.

Codes and standards

- American Association for Marriage and Family Therapy. (2015). Code of Ethics.
- International Association of Marriage and Family Counselors. Ethical Code.

Next: MFT 702 — Theories of Family Therapy